

EMS & HEALTHCARE RECEIVABLES · REFERENCE

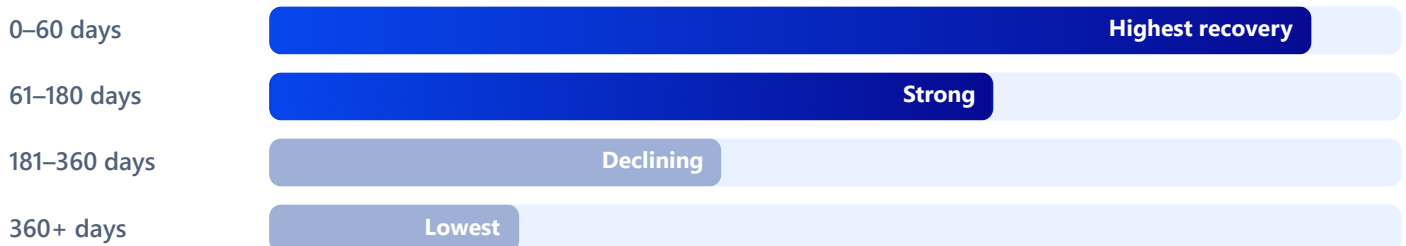
Why Early Placement Recovers More

The age of a receivable is the strongest predictor of whether it is ever recovered

For EMS and patient accounts receivable, the single biggest lever on recovery is timing. The window between the date of service and the day an account is placed determines how much is realistically collectible — and for emergency medical services, where a large share of accounts are already difficult to bill, that window matters more than almost anything else.

Recovery falls as accounts age

Receivables become harder to collect in proportion to their age. Contact information goes stale, payer timely-filing windows close, patients disengage, and documentation degrades. The chart below shows the typical, relative pattern: recovery is highest while the balance is fresh and declines steadily thereafter.



Illustrative and relative — actual recovery varies by account age, balance size, payer mix and contact-data quality. Southwest Recovery does not promise a fixed recovery percentage and will give an honest read on a representative sample before any accounts are worked.

Why the curve drops

- **Contact data goes cold.** Patients move; phone numbers change. The longer an account sits, the harder the right party is to reach — the leading cause of “non-billable” EMS accounts.
- **Payer windows close.** Medicare and commercial carriers enforce timely-filing limits. Unresolved claims that age past those windows become permanently uncollectible from the payer.
- **Patient disengagement.** A recent balance carries urgency and context; an old one feels disputable or forgotten.
- **Documentation degrades.** The supporting record that makes a balance defensible is strongest close to the date of service.

The early-out advantage

An early-out (pre-collection) program works balances in the early past-due window — roughly 15 to 60 days out — as an extension of your billing office, before accounts ever reach hard collections. For healthcare and EMS, that means:

- Address validation, NCOA and eligibility screening so effort lands on good data.
- Insurance verification, resubmission and appropriate appeals with Medicare and carriers.
- Workable self-pay arrangements set up while the balance is fresh — all under HIPAA.
- Fewer accounts that ever escalate, a lower overall cost of recovery, and accelerated cash flow.

What to do with the backlog

Placing early does not mean abandoning older accounts. Aged balances should still be placed — skip-tracing and professional contact recover a meaningful share, and on contingency there is no cost to trying. The point is simpler: the sooner an account is placed, the more comes back. Tightening the placement window on *current* accounts is the highest-leverage change most EMS and healthcare providers can make.

A practical starting point

- Place a test batch of fresh accounts (15–60 days) to see the early-out model work.
- Place the aged backlog in parallel for skip-traced recovery at no upfront cost.
- Measure recovery by placement age — the gap makes the case for placing earlier, permanently.

Put a number on your window

We will review a representative sample of your accounts and give you an honest, age-based read on what is recoverable — and what earlier placement would add.

Call (866) 551-4684 · sdietz@swrecovery.com · swrecovery.com

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